OB-GYN Emergency

AO-03 | Adult Obstetrics

Quick flow

01 Pregnancy / GYN Emergency [EMT / AEMT / Paramedic]

Pregnancy possibility • missed period • bleeding • abdominal pain • recent delivery.

02 Position + Assess [EMT / AEMT / Paramedic]

Left lateral • ABCs • glucose • vitals • bleeding amount • neurologic findings.

03 Shock / Hemorrhage? [AEMT / Paramedic]

Control external bleeding, establish access, resuscitate, and request Paramedic TXA care when indicated.

04 Field Delivery? [EMT / AEMT / Paramedic]

Use Childbirth/Labor; after delivery address uterine atony and postpartum bleeding.

05 Seizure / Eclampsia? [AEMT / Paramedic]

Protect airway • assist ventilation • request Paramedic seizure treatment.

06 Paramedic - Seizure / Hemorrhage [Paramedic]

Magnesium for eclampsia • midazolam for ongoing convulsion • TXA for qualifying postpartum hemorrhage; use doses and safeguards below.

07 Rapid Transport [EMT / AEMT / Paramedic]

Continuous reassessment and early OB/trauma-capable destination notification.

Overview

- Evaluate pregnancy-related and gynecologic emergencies including hemorrhage, ectopic pregnancy, miscarriage, preeclampsia, eclampsia, shock, and trauma.
- Use left lateral positioning, early hemorrhage recognition, seizure control, and rapid destination notification.

Provider-Level Actions

Perform immediately indicated care at the highest authorized level present while protecting both maternal and neonatal physiology.

EMT

- Place the patient left lateral with the right side elevated 10-20 degrees when pregnancy is advanced or supine hypotension is possible.
- Assess airway, breathing, circulation, mental status, abdominal pain, bleeding amount, headache, vision changes, edema, and seizure activity.
- For seizure, protect from injury, support oxygenation and ventilation, check glucose, and request ALS immediately.
- Treat all pregnant trauma patients as requiring physician evaluation and transport.

AEMT

- Perform all EMT care plus IV/IO access and AM-05 shock resuscitation. Support airway and ventilation during seizure and request Paramedic care; midazolam, magnesium and TXA under AO-03 are Paramedic-only.
- Do not delay ventilation, CPR when indicated, or transport for IV/IO attempts; assist the Paramedic with medication preparation within scope.

Paramedic

- Perform all prior care plus cardiac monitoring, midazolam for ongoing convulsion without delaying IM treatment for vascular access, magnesium treatment, TXA for qualifying postpartum hemorrhage, calcium-gluconate rescue for magnesium toxicity, and advanced resuscitation.
- Identify ectopic pregnancy, placental abruption/previa, preeclampsia/eclampsia, and non-obstetric causes while minimizing transport delay.

Indications

- Known or suspected pregnancy with abdominal pain, vaginal bleeding, hypertension, seizure, syncope, or shock.
- Postpartum hemorrhage or uncontrolled vaginal hemorrhage with signs of shock regardless of pregnancy status.
- Pregnant patient involved in trauma or motor-vehicle collision.

Assessment | Preeclampsia / eclampsia findings

- Blood pressure above 140 systolic or 90 diastolic, or a significant rise from the patient's prepregnancy baseline.
- Severe headache, visual disturbance, right-upper-quadrant or epigastric pain, nausea/vomiting, hyperreflexia, or edema of the hands and face.
- Seizure in pregnancy or the postpartum period may represent eclampsia even without a prior diagnosis.

Assessment | Hemorrhage and shock

- Quantify bleeding by pads per hour, visible blood loss, clots, free bleeding, and associated syncope or abdominal pain.
- Consider occult ectopic rupture or placental abruption when shock is disproportionate to visible bleeding.

Treatment steps

- Paramedic: for eclampsia or suspected eclampsia, start magnesium sulfate promptly as the cause-directed therapy. For an ongoing convulsion, use the standardized UP-13 / medication-reference midazolam regimen when immediate seizure control is required.
- After placental delivery, perform uterine massage and follow postpartum hemorrhage treatment.
- A pregnant patient after motor-vehicle collision requires immediate physician evaluation; pregnancy beyond 20 weeks commonly requires 4-6 hours of fetal monitoring.

Medications

Tranexamic acid - Paramedic only

1 g IV/IO over 10 minutes for postpartum hemorrhage with shock when within 3 hours of delivery

- For uncontrolled vaginal or postpartum hemorrhage with signs of shock; follow the indication and timing limits above.
- Postpartum use is contraindicated when birth occurred more than 3 hours before EMS arrival.

Midazolam - Paramedic only

Use the standardized adult-seizure regimen: 10 mg IM when IV/IO is not established, 10 mg IN as an alternative, or 5 mg IV/IO; repeat 5 mg IV/IO once after 5 minutes if seizure continues (maximum total 10 mg).

- Do not delay IM treatment while attempting vascular access during active seizure.
- Magnesium sulfate remains the cause-directed eclampsia therapy.

Magnesium sulfate

4-6 g IV over 20-30 minutes, then 1-2 g/hr infusion for eclampsia or suspected eclampsia

- Primary cause-directed therapy for eclampsia or suspected eclampsia.
- Continuously monitor ECG, BP, respiratory status, SpO_2, and $EtCO_2$.

Warnings / precautions

- For suspected eclampsia, magnesium sulfate is the cause-directed therapy; give midazolam promptly for an ongoing convulsive seizure when needed.
- Magnesium sulfate may cause hypotension and respiratory depression; stop it for clinically significant toxicity and give calcium gluconate 1 g IV slowly with Medical Control.
- Do not use AM-04's undifferentiated-CVA labetalol standing order for pregnancy-related hypertension. Obtain Medical Control for the obstetric indication, agent, dose, and blood-pressure endpoint.
- Do not suggest that a normal ultrasound alone would exclude injury after maternal trauma; appropriate fetal monitoring and physician evaluation are required.
- Vaginal bleeding may be absent in ectopic pregnancy or placental abruption.
- Oxytocin is not on the Claiborne formulary and is not authorized under this protocol.

Clinical pearls

- Maintain a high index of suspicion for ectopic pregnancy in any patient of childbearing age with abdominal pain, syncope, or shock.
- Supine hypotensive syndrome is reduced by left lateral positioning with the right side elevated.

Special populations | Transport

- Use the closest appropriate emergency/OB-capable facility with early notification for hemorrhage, ectopic rupture, eclampsia, severe hypertension, trauma, or shock.
- Pregnancy with any chance of delivery should remain on ground transport and not be placed on an aircraft.

Related pathways / contacts

AO-01 Childbirth / Labor

AO-02 Newly Born

AM-05 Hypotension / Shock

UP-13 Seizure

Source and references

Claiborne County EMS approved protocol manual. | Claiborne County EMS medication formulary, linked protocol pathways, and agency release guidance.

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